INFORMATION TO KEEP WITH YOU

My complete treatment handbook

TPF induction: docetaxel, cisplatin and fluorouracil

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00315
Version 7a

Every 21 days for 3 planned induction cycles.

Treatment stages

01 **Induction chemotherapy**
3 cycles, 21 days apart

02 **Definitive treatment review**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Head and neck cancer: induction chemotherapy

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shivering, sudden illness, unusual bleeding, persistent vomiting, severe diarrhoea, severe mouth pain or inability to drink. Chest pain during or after fluorouracil, sudden confusion, or unexpectedly early severe symptoms needs urgent assessment even if dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing was normal. Report reduced urine, hearing change, ringing in the ears or new numbness promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112 or 999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

What each part of treatment does

This chemotherapy is given before definitive treatment to shrink or control the head and neck cancer. The team explains how induction fits your overall treatment aim and why this particular schedule is appropriate.

TAXANE CHEMOTHERAPY

Docetaxel



Disrupts the cell’s division machinery

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

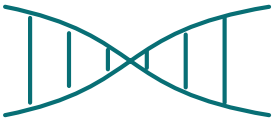
What to watch for

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

PLATINUM CHEMOTHERAPY

Cisplatin

Cisplatin



Damages cancer-cell deoxyribonucleic acid (the cell’s genetic material) (DNA)

Cisplatin links strands of DNA, the instructions inside cells, making it harder for cancer cells to repair damage and divide. Healthy cells may also be affected. Kidney function, hearing, blood counts and nerve symptoms help the team decide whether the next dose is safe.

DNA damage → Cell repair disrupted → Cancer growth reduced

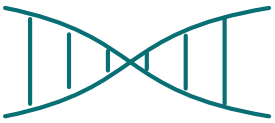
What to watch for

Report reduced urine, persistent vomiting, tinnitus, hearing change or new tingling.

ANTIMETABOLITE CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts production of DNA and RNA

Fluorouracil, often shortened to 5-FU, interferes with the building blocks used to make DNA and RNA, which cells need to grow. It runs through a vein over the prescribed hours. Dihydropyrimidine dehydrogenase (DPD) is an enzyme that breaks it down. Testing for DPD deficiency before treatment reduces risk but cannot rule out every severe reaction.

Cell building blocks disrupted → Cells cannot divide normally → Pump provides prescribed exposure

What to watch for

Report severe diarrhoea, mouth sores, chest pain, sudden confusion, sore palms or soles, or unexpectedly early severe symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis

Keep your diagnosis, treatment name and agreed goal with your records.

Site, type and stage

Ask where the cancer started, what the biopsy shows and how far it has spread. Site, stage, previous treatment and general health affect the available options.

Which pathway applies?

Induction treatment for stage III or intravenous (IV) non-metastatic squamous cell cancer of the head and neck, within a planned course of definitive treatment.

Tumour tests and medicine safety tests

Your team explains any HPV/p16, Epstein–Barr virus or other tumour tests appropriate to your cancer site. Not every site needs the same tests. These are different from blood, kidney and other medicine safety checks.

Understanding the evidence

The NCCP regimen combines evidence and specialist guidance for a defined treatment setting.

A treatment name such as TPF or chemoradiation does not make different dose schedules, patient groups or sequences interchangeable. Ask which research and clinical factors support your exact plan. No single study percentage is presented as your personal chance of benefit.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The stages of your treatment

Treatment stages

01 **Induction chemotherapy**
3 cycles, 21 days apart

02 **Definitive treatment review**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

3 CYCLES, 21 DAYS APART

Induction chemotherapy

Cycles 1–3

Docetaxel 75 mg/m² and cisplatin 75 mg/m² are given on Day 1. Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Induction repeats every 21 days for 3 planned cycles unless cancer progression or unacceptable side effects require a change.

INDIVIDUAL PLAN

Definitive treatment review

After induction and recovery

After induction, the team reviews response and recovery and confirms definitive chemoradiation and/or surgery. The next treatment has its own prescription and calendar; this induction guide does not assign a radiation course or a carboplatin dose.

After induction, the team reviews response and recovery and confirms definitive chemoradiation and/or surgery. The next treatment has its own prescription and calendar; this induction guide does not assign a radiation course or a carboplatin dose.

BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Routine blood safety checks are different from tests ordered for a symptom and from scans used to assess the cancer. Your team supplies exact dates and explains results.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ tests	Before starting: full blood count, kidney and liver tests, and DPD assessment before first fluorouracil unless already tested. electrocardiogram (ECG) is needed for compromised heart function; hearing assessment and measured kidney clearance are arranged when indicated. Discuss hearing, tinnitus, nerve symptoms, swallowing, weight, medicines and allergies.	Full blood count and kidney and liver tests are required before each cycle. Report hearing, tinnitus, nerve, mouth, bowel and fluid-intake changes before dosing; additional tests depend on symptoms and results.	Report symptoms promptly; extra blood or organ tests may be needed.	The team reviews recovery, results and earlier side effects before another dose or phase.	The team follows recovery and response before the next stage, including persistent kidney, hearing, nerve, mouth or bowel effects. Dental, nutrition and swallowing support may continue. Scans, examinations and any radiation-related late-effect tests follow your individual plan; no universal scan or blood tumour-marker schedule is set here.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Treatment and appointments	Confirm the exact regimen, treatment aim, prescription and appointment calendar.	Day 1 includes docetaxel over 60 minutes and cisplatin over 1 hour, plus prescribed fluids, premedication and anti-sickness treatment. Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Total attendance is longer than the drug infusion alone. Confirm inpatient or outpatient arrangements, any pump changes and final disconnection.	Follow the written support plan and record symptoms and medicines taken.	After induction, the team reviews response and recovery and confirms definitive chemoradiation and/or surgery. The next treatment has its own prescription and calendar; this induction guide does not assign a radiation course or a carboplatin dose.	Record the final treatment dates and the named follow-up team.
Mouth, nutrition and swallowing	Agree oral care and assess weight, intake and swallowing. Arrange dental review before planned head and neck radiation.	Review mouth pain, weight, drinking, swallowing and, during radiation, the treated skin.	Report poor intake, choking, weight loss or inability to drink.	Dietitian and speech and language plans adapt to symptoms and recovery.	Continue dental, nutrition and swallowing care as needed.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Tumour assessment and imaging	Confirm the cancer site, stage and tests relevant to that site.	Discuss symptoms and the agreed treatment aim.	Examinations or scans may be arranged for a particular concern.	Response, recovery and your preferences inform the next treatment decision.	Use your individual scan and clinic plan; this regimen does not set one universal scan or blood tumour-marker schedule.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00315 v7a: official regimen · NCI: head and neck cancers · NCI: oral complications of cancer therapies, patient version · NCI: nutrition during cancer treatment

Tests in your plan

Full blood count

WHEN

At baseline and before every induction cycle.

Checks white cells for infection protection, red cells for oxygen carriage and platelets for clotting.

Kidney function and fluid balance

WHEN

At baseline and before each cycle; extra checks or measured clearance when indicated.

Cisplatin can injure kidneys; results and hydration guide safe prescribing.

Liver function

WHEN

At baseline and before each cycle.

Helps assess whether the medicines, particularly docetaxel, can be given safely.

DPD assessment before fluorouracil

WHEN

Before first exposure unless already tested. Moderate or severe kidney impairment can make a blood-uracil phenotype harder to interpret; the team considers genotype testing where appropriate.

Reduced ability to break down fluorouracil can cause severe or life-threatening toxicity. Normal testing does not exclude every severe reaction.

Hearing assessment

If part of your plan

WHEN

Audiology when indicated; report hearing or tinnitus changes before each dose.

Checks hearing changes or ringing in the ears that cisplatin may cause.

Heart assessment / ECG

If part of your plan

WHEN

ECG for compromised cardiac function; additional assessment for symptoms or clinician concern.

Assesses existing heart problems and helps plan safe treatment; fluorouracil-related chest symptoms need urgent review.

Response assessment

If part of your plan

WHEN

Scans and examinations according to the individual treatment plan.

Helps decide the next treatment stage after induction.

THE WHOLE TREATMENT JOURNEY

Docetaxel 75 / cisplatin 75 / fluorouracil 1,000 · induction TPF: treatment and checks

Induction treatment of stage III/intravenous (IV) non-metastatic squamous cell carcinoma of the head and neck, with a separately agreed definitive chemoradiation and/or surgery plan.

Docetaxel 75 mg/m² and cisplatin 75 mg/m² on Day 1, with fluorouracil 1,000 mg/m²/day on Days 1–4. Repeat every 21 days for 3 cycles, unless progression or unacceptable toxicity changes the plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Treatment phase NCCP	Confirm the full multidisciplinary team (MDT) plan and individual treatment calendar	Docetaxel 75 mg/m² and cisplatin 75 mg/m² on Day 1, with fluorouracil 1,000 mg/m²/day on Days 1–4. Repeat every 21 days for 3 cycles, unless progression or unacceptable toxicity changes the plan.	Follow the actual pump/disconnect and support plan	After cycle 3, the multidisciplinary team confirms response, recovery and the definitive chemoradiation and/or surgery plan. No automatic radiotherapy date or universal subsequent regimen is authorised by selecting this induction route.	Record the completed medicines and next follow-up plan
Blood and organ checks NCCP	Full blood count, renal and liver profile; clinical status, medicines, allergy and previous treatment review.	Full blood count, renal and liver profile before each cycle; use the current regimen's exact count and organ-function rules.	Report infection, bleeding, reduced urine or dehydration	Repeat or reassess tests if toxicity or recovery changes the plan	Further blood tests and imaging are arranged by the treating team

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Support medicines NCCP	Docetaxel premedication is dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses should precede treatment unless contraindicated. If oral premedication was missed, the source permits dexamethasone 20 mg IV at consultant discretion. This is not automatic permission to proceed. Reconcile the total steroid with antiemetic dexamethasone; do not add both schedules blindly.	Prescribe cisplatin pre- and post-hydration and electrolytes using current renal/cardiac status and local policy. The national source suggests saline/magnesium with optional potassium, but a reference list is not an individual fluid prescription. Monitor intake, urine output, renal function and electrolytes.	granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) support is required in all cycles; NCCP permits standard or pegylated G-CSF. Prescribe product, dose and timing individually; the regimen does not supply one universal injection schedule.	Reconcile all doses, steroid totals and start/stop dates when the phase changes	Review ongoing medicine need with the team
Mouth, nutrition and swallowing NCCP	Assess mouth, teeth, weight and swallowing needs	Review pain, mouth symptoms, oral intake and hydration	Report inability to drink, severe soreness or weight loss	Dietetic, swallowing and dental support according to need	Continue rehabilitation and late-effect care where needed
Urgent contact NCCP	Save the unit's 24-hour contact and fever instructions	Tell staff about new symptoms or reactions	Seek urgent advice for infection, bleeding, chest pain, severe diarrhoea/vomiting or inability to drink	Do not wait for the next routine appointment if acutely unwell	Keep follow-up and urgent-contact instructions available

What each check is for

1

Induction treatment

Follow your agreed treatment calendar

Shrink and control the cancer before the next definitive local treatment. The team must record the subsequent chemoradiation and/or surgery plan; induction is not the complete treatment pathway.

Ask / act: Confirm active medicines, written supportive-care plan and review criteria.

2

Next-phase / completion review

After the planned course and recovery review

The team checks response and fitness before the next treatment or follow-up stage.

Ask / act: Ask the team to confirm the next phase and actual dates.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Confirm access and training

Your team chooses the venous access device and teaches its care, including a port or peripherally inserted central catheter (PICC) if used.

2 · Before Day 1

Write down the infusion option

Confirm whether your prescription uses one continuous 96-hour fluorouracil infusion or four daily 22-hour infusions; the unit writes down connection, change and disconnection times.

3 · Treatment days

Staff connect and change infusions

Only trained staff connect, program or change the infusion. Do not calculate or alter the rate.

4 · While connected

Watch the line and device

Call for leakage, pain, swelling, damage, disconnection, unexpected completion or a pump that appears not to run. Follow the unit’s device-specific emergency instructions.

5 · At prescribed completion

Attend for disconnection

The actual connection time and selected infusion duration determine completion. Do not disconnect or reconnect the line yourself.

6 · Between cycles

Keep the access-care plan

Keep dressing, flushing and review appointments; report redness, pain, fever or swelling.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Confirm the exact regimen, treatment aim, baseline tests, transport, support medicines and verified urgent contact numbers. Bring a full medicine list and any hearing, nerve or swallowing concerns.

STEP 2

Time in the unit

Day 1 includes docetaxel over 60 minutes and cisplatin over 1 hour, plus prescribed fluids, premedication and anti-sickness treatment. Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Total attendance is longer than the drug infusion alone. Confirm inpatient or outpatient arrangements, any pump changes and final disconnection.

STEP 3

Keeping appointments together

Keep one calendar for blood tests, treatment and any radiation, dental, nutrition or swallowing appointments. If an appointment is cancelled, ask the relevant teams to agree the revised plan.

STEP 4

Practical help

Tell the team about travel, cost, work, caring responsibilities, communication or reading needs. Ask whether a support person or interpreter can attend and how treatment changes will be explained.

STEP 5

Leaving the unit

Check the next blood-test and treatment dates, medicine instructions and whom to call if unwell. Have the service confirm the daytime, out-of-hours and backup contact routes.

Recovery, assessment and longer-term care

The last treatment and the end of recovery are different milestones.

Recovery and continuing effects

The team follows recovery and response before the next stage, including persistent kidney, hearing, nerve, mouth or bowel effects. Dental, nutrition and swallowing support may continue. Scans, examinations and any radiation-related late-effect tests follow your individual plan; no universal scan or blood tumour-marker schedule is set here.

Cancer assessment

Your team sets the examination and imaging plan for your cancer and symptoms. Record who arranges each test and who will explain the result. New or worsening symptoms should be reported rather than saved for the next routine scan.

Information for other clinicians

Keep the regimen name, treatment dates, ongoing medicines and relevant alert information. Tell future clinicians and dentists about head and neck radiation if you receive it, and about any persistent kidney, hearing, nerve or swallowing problem.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Docetaxel 75 mg/m² and cisplatin 75 mg/m² are given on Day 1. Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Induction repeats every 21 days for 3 planned cycles unless cancer progression or unacceptable side effects require a change.

Treatment or check	Timing in this view
Docetaxel	 **Day 1** Docetaxel 75 mg/m² by IV infusion over 60 minutes on Day 1.
Cisplatin and fluids	 **Day 1** Cisplatin 75 mg/m² over 1 hour, with prescribed hydration.
Fluorouracil infusions	 **Days 1–4** Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days.
Symptoms and recovery	 **Days 1–21** Report symptoms at any time. Blood tests and clinical review determine the next dose.

Treatment sequence: Docetaxel 75 mg/m² and cisplatin 75 mg/m² are given on Day 1. Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Induction repeats every 21 days for 3 planned cycles unless cancer progression or unacceptable side effects require a change. After induction, the team reviews response and recovery and confirms definitive chemoradiation and/or surgery. The next treatment has its own prescription and calendar; this induction guide does not assign a radiation course or a carboplatin dose.

Appointments and administration: Day 1 includes docetaxel over 60 minutes and cisplatin over 1 hour, plus prescribed fluids, premedication and anti-sickness treatment. Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Total attendance is longer than the drug infusion alone. Confirm inpatient or outpatient arrangements, any pump changes and final disconnection.

Baseline and ongoing checks: Before starting: full blood count, kidney and liver tests, and DPD assessment before first fluorouracil unless already tested. ECG is needed for compromised heart function; hearing assessment and measured kidney clearance are arranged when indicated. Discuss hearing, tinnitus, nerve symptoms, swallowing, weight, medicines and allergies. Full blood count and kidney and liver tests are required before each cycle. Report hearing, tinnitus, nerve, mouth, bowel and fluid-intake changes before dosing; additional tests depend on symptoms and results.

Recovery and follow-up: The team follows recovery and response before the next stage, including persistent kidney, hearing, nerve, mouth or bowel effects. Dental, nutrition and swallowing support may continue. Scans, examinations and any radiation-related late-effect tests follow your individual plan; no universal scan or blood tumour-marker schedule is set here.

> **Why these bands stay broad:** This 21-day picture describes induction only. Steroid premedication starts the day before docetaxel. Infusion finish times depend on the chosen prescription and actual start time; later radiation or chemoradiation has a separate calendar. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00315 v7a: official regimen ↗

> **There are no guaranteed "safe days".** Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Between-treatment notes

Day 1: Day 1: three-drug treatment begins

Docetaxel and cisplatin are given in the unit with prescribed premedication, fluids and anti-sickness medicines. Fluorouracil begins. Confirm whether your prescription uses one continuous 96-hour fluorouracil infusion or four daily 22-hour infusions; the unit writes down connection, change and disconnection times. Tell the team if steroid doses were missed, you feel unwell or you have new symptoms before treatment starts.

Day 2–4: Fluorouracil continues

Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Only trained staff connect, program, change or disconnect the infusion. Follow the line and pump instructions. Call for leakage, pain, swelling, damage or an infusion that appears to finish unexpectedly or stop.

Day 5–21: Recovery and review

Follow the prescribed home medicine and growth-factor plan. Side effects may continue or appear between visits; no day is guaranteed free of risk. Keep blood-test and review appointments and call promptly if unwell.

Support around the medicines

1 Steroid preparation for docetaxel

The source premedication is dexamethasone 8 mg by mouth twice daily for three days, starting the day before docetaxel, with at least three doses taken before infusion. It helps reduce allergy and fluid retention. Follow your prescribed plan. If doses are missed or you cannot swallow them, contact the unit before attending; the clinician decides the alternative.

2 One anti-sickness and steroid plan

Cisplatin has a high risk of nausea and vomiting. The team provides preventive and rescue anti-sickness treatment. Have all steroid doses reconciled on one list so the docetaxel premedication and anti-sickness prescription do not lead to accidental duplication.

3 Growth-factor support

Growth-factor support (G-CSF) is required with every cycle of this regimen. Your team prescribes the product, dose and injection dates. Confirm who gives it and whom to contact if a dose is missed; it reduces risk but does not remove the need to call for fever or sudden illness.

4 Fluids and kidney protection

Cisplatin requires prescribed fluids before and after treatment. Oral hydration is encouraged for the following 24 hours, adapted to your heart, kidney and swallowing needs. Ask for a practical individual fluid plan and call for much less urine, persistent vomiting or inability to drink.

5 Antibiotics when prescribed

An antibiotic may be added by the treating team according to the regimen and your situation. It is not a medicine to start from an old supply. Confirm the name, start and stop dates and important interactions; infection symptoms still need urgent contact.

6 Food, mouth and swallowing support

Use the mouth-care and pain plan. Report ulcers, infection, poor intake or weight loss early. Dietitian and speech and language support can tailor intake and swallowing advice; arrange dental assessment before planned head and neck radiation.

7  Fertility, pregnancy and other medicines

Discuss fertility, pregnancy, breastfeeding and the medicine-specific duration of contraception before treatment. Bring medicines, supplements and allergies and check before adding over-the-counter or herbal products.

Treatment choices and your priorities

Discuss the whole plan and each stage before consenting.

Benefit and alternatives

This chemotherapy is given before definitive treatment to shrink or control the head and neck cancer. The team explains how induction fits your overall treatment aim and why this particular schedule is appropriate. Ask how the expected benefit and burdens compare with suitable alternatives and with your priorities.

When treatment changes

The clinician may delay, change or stop treatment because of symptoms, test results, response or your preferences. Record which component is affected and what must happen before restarting. Do not restart from an old prescription.

Preparing for and receiving treatment

Plan the appointments and practical support around the exact prescription.

The treatment calendar

Docetaxel 75 mg/m² and cisplatin 75 mg/m² are given on Day 1. Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Induction repeats every 21 days for 3 planned cycles unless cancer progression or unacceptable side effects require a change. This 21-day picture describes induction only. Steroid premedication starts the day before docetaxel. Infusion finish times depend on the chosen prescription and actual start time; later radiation or chemoradiation has a separate calendar.

Recording what was given

Write the actual date, phase, medicines and any change. Record a delayed or omitted dose clearly; it is not automatically made up. The wallet card gives a brief record and the passport keeps the fuller history.

Between visits: symptoms and medicines

Use the written home plan and contact route.

Symptoms have no safe-day guarantee

Effects may begin during treatment or develop later, and some build up over repeated doses. Call for a new, severe or worsening symptom even when a calendar calls the day a recovery day.



One medicine list

Keep prevention medicines, rescue medicines and regular medicines together on one reconciled written list. Record what you take and whether it helps. Do not double a missed dose or reuse an old prescription unless your treating team instructs you.

Mouth, teeth and jaw care

Dental preparation and gentle daily care reduce avoidable problems.

Before treatment

Arrange dental assessment before head and neck radiation. The dental and oncology teams decide what treatment is needed and how much healing time is required; do not schedule extractions independently.

During treatment

Follow the prescribed brushing, fluoride, rinse and pain-relief advice. Report ulcers, infection, new bleeding, difficulty wearing dentures or worsening jaw opening. Avoid starting a new mouthwash without checking if the mouth is sore.

After head and neck radiation

Dry mouth can increase dental problems and some radiation effects last. Keep regular dental care and tell any future dentist that you have had head and neck radiation before invasive treatment. Your radiation/dental team plans any procedure.

Eating, drinking and maintaining strength

The aim is adequate intake and comfort during treatment.

Recognising an intake problem

Record weight and report food or drink becoming harder to manage. Pain, taste change, dry mouth or nausea can each need a different response; a dietitian can adapt the plan to you.

If ordinary food is not enough

The team may suggest food fortification, supplements or tube feeding when intake or swallowing safety requires it. A tube is a support option, not a judgement about how well you have tried to eat. Training and follow-up are needed.

Fluid needs are individual

Ask for a realistic drinking plan that accounts for kidney/heart problems and any feeding tube. Persistent vomiting, dizziness, markedly reduced urine or inability to drink needs urgent contact.

Swallowing, voice and communication

A speech and language therapist can help protect function.

Swallowing assessment

Report coughing or choking with food or drink, a wet-sounding voice after swallowing, food sticking or repeated chest infections. The team may need to assess aspiration risk and safe textures.

Exercises and rehabilitation

Do swallowing or jaw exercises only as taught for your situation. Treatment and recovery can change what is comfortable and safe. Ask for reassessment when symptoms change.

Communication needs

Tell the team about changes in speech, voice or hearing. Ask for written instructions, communication aids, an interpreter or help involving a support person.

Daily life and emotional support

Practical adjustments can make the plan easier to follow.

Energy and activity

Balance rest with manageable activity according to your ability and team advice. Ask for help if weakness, falls, sleep difficulty or pain interferes with daily life.

Smoking and alcohol

Ask for support to stop smoking and discuss alcohol with the team. Mouth irritation, medicine interactions and head and neck recovery can change what is safe for you.

Emotional and practical concerns

Tell the team about anxiety, appearance or intimacy concerns, finances, travel and caring responsibilities. Ask which local supports are available; resources can be added to your record when confirmed.

Your records and additional help

Keep essential instructions available on paper.

The passport and wallet card

The three-page companion gives the essentials. Use the passport for the agreed plan, visits, test results, symptoms and questions. The wallet QR opens this exact treatment’s further information; it contains no personal clinical record.

Your urgent contact plan

Have the treating service enter and verify daytime, out-of-hours and backup numbers before relying on this pack. Keep the written contact and temperature instructions where you and a support person can find them.

If you prefer paper

Use the full printable handbook in its chapter order and write questions in the passport. You do not need to scan the QR or follow an internet link to use the essential treatment and safety information.

Questions worth asking

Are all TPF or TCF schedules the same?

No. This is NCCP 00315: docetaxel 75, cisplatin 75 and fluorouracil 1000 mg/m², using the exact days and infusion times in this guide. There are 3 planned induction cycles. The name alone is not enough to identify the prescription.

When does the fluorouracil finish?

Confirm whether your prescription uses one continuous 96-hour fluorouracil infusion or four daily 22-hour infusions; the unit writes down connection, change and disconnection times. Do not disconnect or change the infusion yourself.

What if I miss the steroid tablets?

Call the treatment unit before docetaxel. The clinician decides whether an alternative premedication or a change to the appointment is needed.

Does a normal DPD test mean fluorouracil cannot cause serious effects?

No. Testing reduces risk but cannot exclude every severe reaction. Unexpectedly early severe diarrhoea, mouth pain, illness, chest pain or confusion still requires urgent assessment.

> **Try explaining the plan back:** Can I explain which treatment phase I am in and how many induction cycles are planned? Do I know when to start steroid tablets and what to do if a dose is missed? Can I explain my exact fluorouracil infusion and disconnection plan? Do I know my growth-factor plan, urgent symptoms and verified out-of-hours contact route?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction Day 1 · docetaxel / cisplatin / fluorouracil

Active medicines: Docetaxel + Cisplatin + Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Ondansetron	16 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Olanzapine	5 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.

Medicine	Dose / route	When
Dexamethasone for docetaxel premedication	8 mg PO BD	For 3 days starting the day before docetaxel; at least 3 doses before treatment unless contraindicated Source-linked draft; reconcile to one authorised prescription before use.
Aprepitant	80 mg PO OD	Days 2-3 Source-linked draft; reconcile to one authorised prescription before use.
Metoclopramide	10 mg PO TDS	Subsequent days / as directed Source-linked draft; reconcile to one authorised prescription before use.
Olanzapine	5 mg PO OD	Days 2-4 Source-linked draft; reconcile to one authorised prescription before use.
Reconciled corticosteroid schedule	Prescriber/pharmacist to complete As prescribed As prescribed	Include docetaxel Day –1/Day 1/Day 2 coverage and delayed antiemetic needs; no duplicate dexamethasone Source-linked draft; reconcile to one authorised prescription before use.

Additional phase-specific support

- Docetaxel 75 mg/m² and cisplatin 75 mg/m² on Day 1, with fluorouracil 1,000 mg/m²/day on Days 1–4. Repeat every 21 days for 3 cycles, unless progression or unacceptable toxicity changes the plan.
- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) support is required in all cycles; NCCP permits standard or pegylated G-CSF. Prescribe product, dose and timing individually; the regimen does not supply one universal injection schedule.
- Docetaxel premedication is dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses should precede treatment unless contraindicated. If oral premedication was missed, the source permits dexamethasone 20 mg intravenous (IV) at consultant discretion. This is not automatic permission to proceed. Reconcile the total steroid with antiemetic dexamethasone; do not add both schedules blindly.
- Prescribe cisplatin pre- and post-hydration and electrolytes using current renal/cardiac status and local policy. The national source suggests saline/magnesium with optional potassium, but a reference list is not an individual fluid prescription. Monitor intake, urine output, renal function and electrolytes.
- Document dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) phenotype and/or genotype before first fluorouracil exposure unless previously tested. Complete deficiency precludes fluorouracil; partial deficiency needs an individual starting-dose plan. Renal impairment can elevate uracil and misclassify phenotype; consider genotype. A normal result does not exclude all severe toxicity.
- Docetaxel premedication is dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses should precede treatment unless contraindicated. If oral premedication was missed, the source permits dexamethasone 20 mg IV at consultant discretion. This is not automatic permission to proceed. Reconcile the total steroid with antiemetic dexamethasone; do not add both schedules blindly.
- Arrange dental/oral assessment, mouth care, dietetic and swallowing support according to need. Review weight, oral intake, pain and hydration throughout chemotherapy and radiotherapy; no automatic feeding-tube decision is generated.

- Preserve the CUH high/moderate and docetaxel proforma adjuncts where applicable: omeprazole, local nystatin/chlorhexidine mouth care, breakthrough antiemetics and loperamide. These are historical local reference items requiring current-policy and patient-specific review, not universal prophylaxis or an additional duplicate prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Induction Days 2–4 · fluorouracil continues

Active medicines: Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Continuation/delayed antiemetic prescription	Prescriber/pharmacist to complete As prescribed As prescribed	Days 2–4 fluorouracil and two days after systemic anticancer therapy (SACT), accounting for delayed Day 1 cisplatin risk Source-linked draft; reconcile to one authorised prescription before use.

Additional phase-specific support

- Retain prescribed delayed cisplatin antiemetics; no new docetaxel/cisplatin infusion.
- Fluorouracil: continuous intravenous (IV) infusion over 96 hours on Days 1–4; the source also permits daily 22-hour infusions on Days 1–4. Record the actual selected option, pump/bag changes and disconnect time.
- Document dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) phenotype and/or genotype before first fluorouracil exposure unless previously tested. Complete deficiency precludes fluorouracil; partial deficiency needs an individual starting-dose plan. Renal impairment can elevate uracil and misclassify phenotype; consider genotype. A normal result does not exclude all severe toxicity.
- Arrange dental/oral assessment, mouth care, dietetic and swallowing support according to need. Review weight, oral intake, pain and hydration throughout chemotherapy and radiotherapy; no automatic feeding-tube decision is generated.
- Preserve the CUH high/moderate and docetaxel proforma adjuncts where applicable: omeprazole, local nystatin/chlorhexidine mouth care, breakthrough antiemetics and loperamide. These are historical local reference items requiring current-policy and patient-specific review, not universal prophylaxis or an additional duplicate prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

After induction · multidisciplinary team (MDT) selects definitive treatment

Active medicines:

Emetogenic risk: Minimal

Source-linked clinician/pharmacy draft. Confirm active phase, doses, routes, steroid totals and start/stop dates; no administration authority.

Additional phase-specific support

- After cycle 3, the multidisciplinary team confirms response, recovery and the definitive chemoradiation and/or surgery plan. No automatic radiotherapy date or universal subsequent regimen is authorised by selecting this induction route.
- Select the separately prescribed regimen when it is known; this review phase does not authorise further systemic anticancer therapy (SACT).

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hair and scalp
Docetaxel
Hair thinning or loss can occur. Ask about scalp care and practical support.

2 Ears and hearing
Cisplatin
Ringing or hearing loss can persist (ototoxicity). Report a change before the next dose.

3 Mouth and throat
Docetaxel + 5-FU
Soreness or ulcers (mucositis) can make eating and drinking difficult. Contact the team for pain or poor intake.

4 Heart and chest
5-FU
Chest pain can signal a heart effect (cardiotoxicity). Seek urgent assessment during or after the infusion.

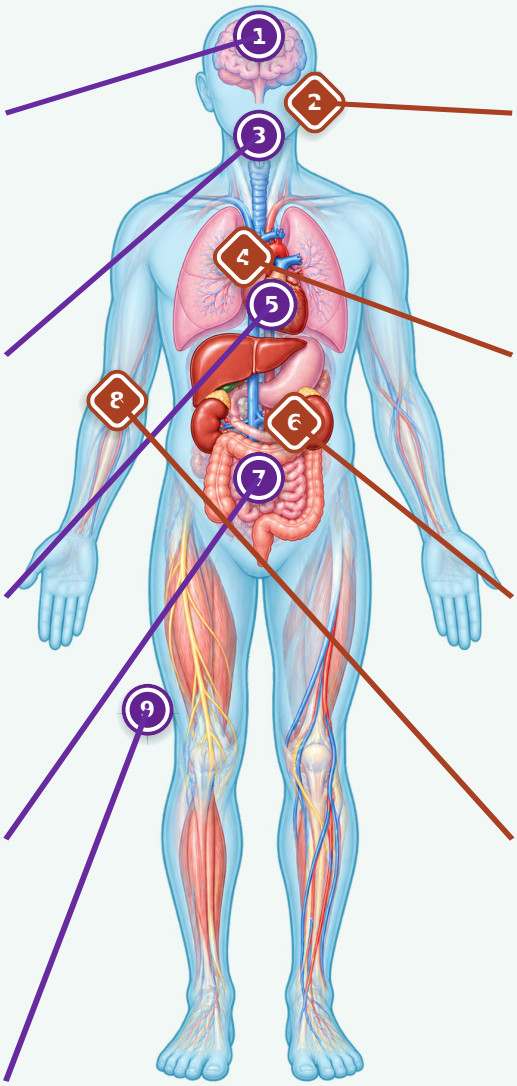
5 Blood and infection
Docetaxel + cisplatin + 5-FU
Low white cells can cause serious infection; low platelets cause bruising or bleeding; anaemia causes fatigue. Call for fever or sudden illness.

6 Kidneys and fluids
Cisplatin
Kidney injury and mineral changes can occur. Prescribed fluids and blood tests help protect you; report reduced urine or inability to drink.

7 Stomach and bowel
All three chemotherapy drugs
Nausea and vomiting can occur; 5-FU may cause severe diarrhoea. Persistent symptoms can cause dehydration and need prompt help.

8 Skin, nails, hands and feet
Docetaxel + 5-FU
Docetaxel may affect nails and retain fluid; 5-FU may cause painful palms or soles (hand–foot syndrome). Report swelling, pain or skin breakdown.

9 Hands, feet and nerves
Docetaxel + cisplatin
Numbness or tingling (peripheral neuropathy) can persist. Report difficulty with buttons, walking or balance before another dose.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shivering, sudden illness, unusual bleeding, persistent vomiting, severe diarrhoea, severe mouth pain or inability to drink. Chest pain during or after fluorouracil, sudden confusion, or unexpectedly early severe symptoms needs urgent assessment even if DPD testing was normal. Report reduced urine, hearing change, ringing in the ears or new numbness promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112 or 999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00315 v7a · pNCCP 00315: treatment/supportive-care sections; NCIS v8 pp4–5**
   https://healthservice.hse.ie/documents/6956/315_DOCEtaxel_CISplatin_5FU_TPF_1.pdf
2. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **eviQ: port**
   https://www.eviq.org.au/p/507
6. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
7. **eviQ: pump**
   https://www.eviq.org.au/p/502
8. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
9. **NCCP 00315 with current NCIS and local supportive-care policy**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf